

Saturday, October, 22, 2022 48:50

Interviewer

Good morning! How are you?

Thank you again for participating in this interview. Again, we *do* believe your experience and perspective would bring much to the project.

Before we begin, is it alright if we record this interview? It will just be for transcription for note-taking, but I will not be keeping the video.

Therapist 00:00

I'm okay with it.

Interviewer 00:03

No problem at all. We won't make you a YouTube star yet. All right. And just to start with as well, [Name], I just wanted to get a better sense of your clinical practice. First, I wanted to clarify what your primary modality for couples therapy is.

Therapist 00:22

Yeah. I think it's a combination. I'm I'm quite long in the field, so as a [occupation]. Also working with couples. So I'm a trained Gestalt therapist. And I did emotional focus therapy, the whole training. But I didn't do the process. But it's a long process to get recognized EFT therapist, but I use a lot of that. But I have also to say that some Gottman tools I did, I read a lot of stuff. I think I'm also a somatic experience practitioner. So all the trauma work and the body is part of my modality to work with couples.

Interviewer 01:15

Interesting. That's very interesting. How long have you been working as a couple therapist now?

Therapist 01:27

Good question. But I assume it has to be, I think [11-15] years.

Interviewer 01:36

[11-15] years. All right. And are there certain....

Therapist 01:40

[11-15] years, something like this?

Interviewer 01:45

[11-15] years. All right. And are there specific types of couples that you provide your services to?

Therapist 01:50

No, no.

Interviewer 01:52

All right. And where are you currently based now? Which country are you based?

Therapist 01:58

At the moment, I'm in [City 1], actually, but I'm based in [Country]. But I'm in the transition phase back to [City 1]. So it's. It's something between. So I work the last [1-5] and a half years in [Country] and I'm on my way to move back to [City 1] and open my practice here again.

Interviewer 02:21

Interesting. So was that were you in [City 1] when you were doing. Basically in the [11-15] years that you worked as a couple therapist?

Therapist 02:32

In [City 1] and in [Country]. So I had my own practice in [Country] as well. It was a holistic mental health practice and I worked there, which was quite interesting because you have a very different social background there with the couples and all the individuals and and the rest. I worked in [City 1]. Yeah.

Interviewer 02:56

Interesting. So. Your services are now that you're currently based in [City 1]...Do you give the couple's therapy both to people based in [Country 2] and in [Country]? Or is your practice now just based off giving services to couples in both?

Therapist 03:16

Both. I see couples in [City 1] a in-person, and I still see couples online in [Country].

Interviewer 03:29

All right, I'd really love to get like to dive deep into the possible differences between your clients in [City 1] and [Country] later on. But now I just like to ask for now, when your clients come to you, what usually is the presenting issue or the reason that they seek your service?

Therapist 03:48

Do you want? Do you mean what? Why they are calling me or do you mean what I think the issue is? Maybe there's a difference.

Interviewer 04:06

What do they say when they call you? And what do you think it is? Really?

Therapist 04:11

Okay, So I think often they call me because they have some issues. So a betrayal or often I think it's interesting because when I think the couples I have at the moment and it's different every time, but the moment I have women wives who complain about their

husbands, that they are emotional not available. But I also have the other side when when wives are kind of disconnected already because the right everything to catch him and and he he's desperate because she is not I think most of the time what I see is just the disruption. No, what is the English word? I don't find the English word. So the connection is disconnected. Yeah. So this is what I see. I have the feeling they are not connected anymore or not as deep as they had been. And for me it's often about the connection, about being in contact with each other.

Interviewer 05:34

Interesting. And are these issues that they usually come to you with? Do these tend to differ between [City 1] and [Country]?

Therapist 05:45

On the surface, yes. But I think that relationship is work unique worldwide. It's about being seen. Being heard. Being in contact. And I think that's worldwide in every culture. The same stuff.

Interviewer 06:08

Interesting. Interesting. Yeah. On the surface, what are the surface differences?

Therapist 06:16

In [Country], it's often when you work with [Nationality] background or [Nationality] background. You have often the family involved. So. So it's the mother in law, the father in law. The family is involved and often couples have issues regarding this so that the mother in law has too much saying in the family system or that. The partners, both men or women, don't have the feeling that the couple's relationship is the base. They always have the feeling that the parents in law interfere with it. Or often, for example, I have a [Nationality], a couple with her first attachment figure is her father and not husband. And he knows it and feels it and she declines it. But it's obvious. So it's I think this is really a big difference regarding the topics because something like this is rare in [City 1]. Yeah.

Interviewer 07:26

Interesting. That's really interesting. So when the clients come to you, do you do a formal assessment?

Therapist 07:35

Um, I used to do, but actually, I don't do anymore.

Interviewer 07:41

Why not? That's interesting.

Therapist 07:45

Because I use the first session to, to get the information, so I used to give them. Queries. Sorry. Today I my English is I do not know. I did so many sessions in [Language] so to fill out stuff. Yeah. So this is what I did. Yeah. My experience is then I have the information, but I don't have the emotional connection to it. So I prefer to ask people things and to be in contact and to be present and catch all the informations I get when I ask the person a question.

Interviewer 08:32

Interesting. And when you used to do it before, what were the skills that you were using?

Therapist 08:38

There are some EFT skills, actually. And primary. I used them. Yeah.

Interviewer 08:46

Mm hmm.

Therapist 08:48

And I taught them in the training. So I don't know if this is something official, but.

Interviewer 08:53

Do you mind if I ask? Did you pay for those skills?

Therapist 08:57

No, no. I mean, it depends because I think one was in a book. So yes, I read the book, but no.

Interviewer 09:05

And you didn't use any of the Gottman skills?

Therapist 09:10

Not really. To be honest.

Interviewer 09:14

Do you remember? Sorry. Do you mind if I ask if you remember? What were those in the EFT skills that you were using? What were the topics within the assessment?

Therapist 09:23

And I think there's a Deutch man interview. And I think. And I mean, the attachment theory is something where my whole book is based on not only from the EFT side, because I my bachelor thesis about attachment theory. So but back to your question. I'm sorry. I really cannot remember. It's a device that I use. I mean, I have a look in my.

Interviewer 09:57

Aside from not feeling the emotion about it or the emotional weight of the information, were there any other challenges when you were using these skills?

Therapist 10:15

Let me see. For me again, I think that I am. My entire work with people is to be very present, to be in contact, to offer relationship. So I what I get is more information when I ask things because sometimes, you know, people don't have the words to describe something. They don't know if they feel something or don't feel. Even some people have difficulties to name emotions. So what? They don't have a very variety of emotions. They know good or bad. Black and white. But and therefore, I think for me, it's. It's too narrow to use a scale of. Because I have the feeling that I can get more information when I when I. I'm in contact with the people and I'm and in a way, I use the scale because there are some questions I always ask.

Interviewer 11:31

Yeah. Interesting.

Therapist 11:33

It make sense. So it might not be on paper, but there are some questions I always ask. Yeah, because I get a lot of information and I like to collect information in a wider field to see anything in relationships, in couples therapy. It's quite good because you, you get the information from the person you ask, but you also get the reaction of the partner. So it gives you a lot of information when you're doing so...

Interviewer 12:11

And these questions that you always ask include what?

Therapist 12:18

Good question. I'm not so prepared because I think I'm very automatically Give me a second, I will figure it out. I ask things about attachment. So my first question is when something happened in your life, something meaningful, who would you contact? Yeah, this would be one question I ask. I ask also about the past when something meaningful, meaningful happened in your life and you were little. Who was the first new contact. So you get a little bit of feeling about the attachment pattern of a client to understand was there somewhere or not? So this is one of the questions. Another question I always ask clients what is working? What is good in the relationship? I ask clients things like when was the last time they felt connected? Yeah. I ask all this how they met and if they can remember how it felt the first weeks of the relationship. So these are more or less the questions I ask.

Interviewer 13:53

Yeah, that's super interesting. I wanted to ask as well, and I think I've touched upon this in the email, but across the years, different therapists and different researchers have

characterized the construct of relationship quality quite differently from each other. I wanted to get your view on it for you. What is the relationship quality?

Therapist 14:23

So you want me to describe what I think? Okay. I mean, there are certain things I think there's the ability to have empathy and compassion with each other. I think there's the. Also the ability to set boundaries, the ability to resolve conflicts, and to understand that a conflict is part of a relationship and the conflict is the opportunity to know each other better. So if a couple comes and they say we never have a fight, we never have an argument. But we are disconnected. And I think no wonder. So, yeah, for sure it's about communication. Not only to articulate things, it's also about to listen and to understand that it's a little bit like to understand it's sometimes not against the partner, it's for me. If I say I don't want to do so, this kind of boundary line and understanding compassion reflection things and I trust and honesty is for sure. So. And I also think that a couple if could understands that it's to fall in love is one thing, but to building a relationship, to building understanding is something very different. Yeah. And it's it's a process and it takes time sometimes. And sometimes it's easier and sometimes it's not so easy. This, I think, is a very important relationship. Quality that both. Hopefully understand that. Taking responsibility. What a relationship. So if only one partner is taking responsibility for the relationship, the other one is not. Is is difficult. And that both have space so that it's not clear. But it depends some partners on some couples need more of this but still I think that everybody needs space this is a quality. And yeah, for sure, in a love relationship, it's intimacy.

Interviewer 17:21

All right. So would you say that these are the sort of skills that differentiate couples with high relationship quality and with low relationship quality?

Therapist 17:33

So I thought, what was the question?

Sorry. Because I think forgiveness. Forgiveness is also something people need to forgive each other. So what was the question?

Interviewer 17:48

No problem. The question is for you what differentiates couples with higher relationship quality and with those with low relationship quality? Would you say it's a difference between having these skills that you mentioned?

Therapist 18:02

I think when you're having two skills, then both having two skills, it's they have an easier life. It's less stressful. Yeah.

Interviewer 18:15

So you so if I can clarify, it's like if both have these skills, it's what differentiates those with couples with high relationship quality and with low relation.

Therapist 18:27

Yeah. Good question. I think. ideally both have the skills or I don't think that I think that humans are so unique. Yeah. That you have some skills and some and I think it's a process to learn from each other. So I think it's not black and white that both needs the skills or that one has the skill, another has no skills. Maybe some is good in empathy and compassion and the other one is good in communication. So I think it's it's every couple has its own cocktail in a way or combination.

Interviewer 19:10

Yeah. All right. That's super interesting. Every every couple has their own cocktails and their own constellation because actually, I wanted to ask you as well, like, for you, what do you think is the most important things in a high quality relationship? Would you say it's this like all the skills that you listed or would you say that there are what would you say are the most important things?

Therapist 19:41

In a way, I think. But this is also what I personally think is the most important thing. Personally is to be curious, to have your curiosity. To be curious about each other and. This is things I said that are important, but in a way to be curious what is going on. Because I think what we all want, we want to be seen and heard and we want to have emotional resonance. And therefore, I think it's about curiosity. Yeah. So I can ask, how was your day? Or I can be curious about how was your day? Yeah, and it's a very different quality and this great connection for me. And then I can and I think this might be for me, one of the most important things. Yeah. To have this curiosity coming with all the other things, to have compassion, to have empathy, to do know how to self regulate coming together. But I think for me, one of the main skills is to have an open heart and to be really curious about the other person.

Interviewer 21:08

That's really interesting because I wanted to ask as well, you know, we have these frameworks that we use in couples therapy. Of course, you have your own views as well about relationship. I wanted to ask just how you conceptualize the relationships often differ from how your clients conceptualize your relationships. And does this impact therapy?

Therapist 21:37

What is the different? Help me to catch the answer again. So the question is if how I shape my relationship has an impact.

Interviewer 21:56

Does how you think about or that's how you conceptualize your relationships. That's how you think about relationships often differ from how your clients think about or how your clients conceptualize relationships and that impact therapy.

Therapist 22:13

I still don't get the question. I'm very sorry. I'm very sorry.

Interviewer 22:18

No problem.

Therapist 22:19

What? What would you like? Yeah. So what is the question? If how I do relationships.

Interviewer 22:26

Or how you how you think about relationships. When you think about relationships, you have a certain probably like a schema. Or like how and would that differ from the schema of your clients about relationships?

Therapist 22:43

Yeah. Yeah. Sometimes it is different, sometimes it's different. And I think that that's the reason why they are in therapy sometimes although to understand more about relationships, not only in couples therapy, although in individual therapy, people want to understand relationships of.

Interviewer 23:02

Usually what's the difference and does this impact get therapy? Does this impact how you give your services?

Therapist 23:10

I'm pretty sure it impacts because therapy is something very personal and for sure it impacts. How I see relationship. Maybe not how I'm, how good I good I am in relationships. I don't know but still I think Yeah of course it has an impact because how I see relationships or what I collected what does it mean to be in a good relationship has an impact how I see and understand someone others relationship and how I do therapy for sure. Because. And yeah. For sure. It does. And I also think, for example, that it's important that the therapist has relationship skills, because if a therapist doesn't have relationship skills. And has, I don't know, the tendency to cut people out of the life or cannot forgive or blah, blah, blah, how should this therapist work with couples? Does it make sense? So I think one of the things is, is a therapist that you have to it's necessary to work on your own relationship skills in order to offer good therapy to others.

Interviewer 24:34

Hmm. That's interesting. All right. That makes sense. That makes total sense. I wanted to ask as well whether we know that the relationship is a complex system. It involves love, sex, identity or relationship maintenance, behaviors, satisfaction, conflict, etc. during therapy. How do you decide which one to focus on in this complex system? How do you decide which one to to in the complex system to target?

Therapist 25:09

It depends. Sometimes clients tell me what is the most important thing for them. So they want to improve one or the other. And then I'm as a Gestalt therapist, I'm working with the phenomenon. So I'm I'm watching what is in the room. So be very conscious of what is going on. And I shared with my clients. So I say, I see. How would it be to work first on these? But I think this would be important. But I think it's about collecting what the couple presents and collected. And often it doesn't matter which door you use. Yeah it has to be right for the couple, not for me. So it's important that the couple says, yeah, this is something we would like to understand better to work on it.

Interviewer 26:13

Yeah. Interesting. And then how do you then determine when to end couples therapy?

Therapist 26:22

How did to avoid what?

Interviewer 26:23

Sorry to end the couples therapy. To terminate it. To finish?

Therapist 26:28

It depends. Sometimes the couples are in a much better place. We would like to end it. Sometimes I have the feeling that they are much better placed and it might be time to end it. Sometimes a goal is achieved, but sometimes also to be honest the couple has the feeling it doesn't work. The goal is not achieved. Or maybe it's achieved because they don't need couples therapy anymore, because they don't want to be together anymore. Because the purpose of couples therapy is not to stay together.

Interviewer 27:03

All right. And usually for these successful cases, when the treatment goals have been achieved, you're satisfied. They're satisfied already. Usually. How long does it take?

Therapist 27:21

Good question. But I would say at the end of 15 sessions approximately.

Interviewer 27:34

And your sessions are off. How often? Once every week. Once every two weeks.

Therapist 27:41

I try to do couples therapy. I offered once every other week, but I really have although some couples they only have time once a month. So I'm not stubborn about this because I think about the client and not about me. They will find out.

Interviewer 28:08

And the sessions last. How many? How long one are?

Therapist 28:14

90 minutes.

Interviewer 28:15

90 minutes. All right, All right. Okay. So we are actually I think I touched upon this in the email as well, that we are currently developing a relationship quality scale that hopefully better represents the relationship as a complex system that aims to help therapists both in treatment planning and in treatment evaluation. How how interested would you be in a scale like this?

Therapist 28:50

I think it's a quite helpful tool. Maybe more useful for younger therapists and also useful for teaching therapy, I think quite useful. I don't know if I personally would use it because I'm I'm quite long in the business and to be honest. But again, I think it's a useful tool and it might be helpful. It might be helpful to have a skill to communicate with other therapists. I don't know how how it's planned to do how how many points somebody has on the scale and to have another better understanding sometimes I think that might be helpful. Yeah. So. I mean, it's interesting because. I often we don't mean the same when we talk about relationship, aren't we? So that's quite helpful to have a skill to have a common understanding.

Interviewer 30:05

Hmm. And when I know personally, of course, you said that it might be useful for younger therapists, for teaching therapy, etc., But for you, you know, you might not read it because you're very tenured in your career already. But if ever I just wanted to ask you, like, how can we make sure that this scale is useful in, in therapy? You know, like, what would you be looking for? Yeah, potentially in a newly developed scale, how can we make sure that it's useful for someone like you?

Therapist 30:54

Is this scale planned to have it as something the therapist can ask the client? Or is this scale meant to to give the client to fill it up.

Interviewer 31:06

More along the client to fill it out potentially before therapy, if not during the session, but before they come in. And then it will potentially what I can imagine is it will already give an automated report for the therapist. This is a discussion like that.

Therapist 31:28

I think I'm not the right person to give you an answer. Yeah, I'm not a [Occupation]. I'm a [Occupation]. Yeah. So I'm not very familiar with scales. As I said, I don't use them a lot. It's for sure a proper tool, but I don't have an idea. I think for me, a skill has to be not too long because I want to have my clients to sit, write down a lot of things. Again, I'm not the right person to to to be asked. But I was thinking it might be easier to have something. You just have to make your cross and not to think about it because again, people are scared when they do therapy. This is something very personal. So if you give them sometimes a scale and they have to fill it out, they are stressed too to name things or sometimes even don't know how to name things. Yeah. So for me again is I know I might not the right person, but for me it would be more a scale to say, okay, from 1 to 10. How good is blah, blah, blah. So then it's easier sometimes for clients. I think again. I think I'm not the right person for for this.

[Portion Redacted]

Interviewer 34:27

Yeah. Yeah, that's that's a great information to have. Yeah. All right. And I can imagine your answer as well, but they need to ask anyway anyways, because you have to do that by being interviewed though. But would you be interested in further collaborating to further development of the scale? Um.

Therapist 34:48

Again, I don't think that I would be the right person for that, but it's quite interesting. I'm happy that people do such things because it's really important and it's, it's a really awesome work you do, but I might not be the right person.

Interviewer 35:05

All right. No problem at all. Actually, we were just wrapping up. We will be wrapping up now. So I just like to have a very quick overview of what they have been talking about or what we have been discussing during the session. First, I wanted to get a better sense of your clinical practice. So you said that it's really more of a combination for you, what your practice is just all emotion focused therapy. You've done quite a bit of learning from Goldman as well. You read a lot of stuff. You do trauma work, you read stuff from trauma, like working with the body, and you've been a couple therapists from around [11-15] years now. It was in [City 1], then in [Country]. Then [1-5] years ago you transitioned back to [Country]. So you give a couple therapy to both [City 1] in person and to [Country]ans online currently. So when I ask you, when do what.

Therapist 35:59

I was the last [1-5] years only in [Country]. I was in [City 1] my whole life and the last [1-5] years in [Country]. And now I'm in my transition back from [Country] to [City 1].

Interviewer 36:12

Oh, okay. Just this year.

Therapist 36:14

Just this year? Yeah.

Interviewer 36:17

I thought [1-5] years ago you moved back. All right, that's super interesting. All right, so when I ask you what when clients come to you, what usually is a present, an issue? You said that on the surface level, you know, it's issues of betrayal, issues of emotional unavailability, disconnect that for [Nationality], the topics can, can can be related to family as well, the mother in law. The other attachment figures are they seem to be more primary than the husband or the wife. But you think like deep down it's really like Universal is being seen, being heard, being in connection with each other. But aside from that, I wanted to maybe like circle back on that as well. Like in your practice, are there other differences that you see from couples in [City 1] and in [Country]? Like what is it like? Like, are there major or are there significant differences giving couples therapy in [Country]? And now that again, you're in [City 1].

Therapist 37:23

Again, I'm a big believer that relationship is the same worldwide doesn't make ...or something. So I think that the core is the same. It might be a little bit difference. I mean, the topic sexuality or the topic Responsibility to parents or family or the topic same or emotions are different. Yeah. And then again, it depends if you work with Indian couples or Chinese couples. So we have the population is Chinese, Chinese, mostly Chinese. And there's also a difference.

Interviewer 38:10

What would that be? What were the difference between the [Nationality] and [Nationality] couples?

Therapist 38:15

At least many..Of course they are. I cannot generalize it, but. With a [Nationality] background, you are more disconnected from your emotions.

Interviewer 38:33

What sort of connected or disconnected?

Therapist 38:34

Disconnected from your desire. Not to feel. So they have no names for feelings. I know that there is a lot of shame and once a client told me that they don't cry on a funeral, they hire

some people to cry for them. And this is something. Like what? Yeah. As a this is something for us. Western is really not understandable. Like what? So they keep the emotions very close. And then you have of course, there's a different different. It's not a different way, but I think it's necessary to understand that and to see that.

Interviewer 39:21

Interesting. And for [Nationality], would there be an [Nationality]?

Therapist 39:26

They have much more compared to [Nationality] people. They have much more whatever. Not sensibility. And they have more Just have to I have to Google this word. Sorry. The right word.

Interviewer 39:46

No problem.

Therapist 39:48

One second.

Interviewer 39:50

No problem.

Therapist 39:51

So that they are all different sensuality. Couples have much more sensuality than [Nationality] couples, for example. And ability to name emotions.

Interviewer 40:19

But you also mentioned as well like sexuality, that the differences in topics of responsibility, family, shame, sexuality between [Nationality] and [Nationality], for example, couples sexuality in terms of what what would be the difference?

Therapist 40:37

You mean sorry.

Interviewer 40:39

You mentioned earlier that there might be differences, for example, in responsibility and family and sexuality and shame in terms of sexuality. Like, what would be the difference between [Nationality] and [Nationality]?

Therapist 40:54

I think there's a lot of shame about body and generally the the sensuality. But also the sexuality in a way. So there's a lot of shame. There's not open then. It's. [Nationality] are often in the mood. I have to instead want to. Yeah, so I have to have sex. We know where

that goes. Instead of I feel I want to have sex. So Friday night we have sex because we always have sex at Friday night. Yeah, something like this. Again, I cannot generalise that. Yeah, I wouldn't say that. And not all [Nationality] are like this. Of course not. But there's a tendency to have not this playful ...sensitivity, you build something up in touching each other. Maybe when they are young, but they don't know young, ..I see them when they are married and when they have some difficulties, but they have not. Maybe the bodily connection. They don't have.

Interviewer 42:20

Interesting. All right. And then later on, we talk about assessment as well. You used to do it before. Especially with the PhD skills. You don't do it anymore. In your experience, you get more information than you do it, like in person interviewing them. Especially because when you use the skills, they don't have the emotional weight. You have the information, but you don't have the emotion about it. And for you, it's really like your work is to be really present. So, you know, and you think it's more informative because, as you said, people have difficulties gaining emotions. They might not know. Some people, sometimes people don't have the words. So for you, like a scale, narrow is too narrow. So for you, it's really about interviewing them during the session, about their attachment, about when was the last time they felt connected and so on. Talking about relationship quality, you also, when asked for you to describe what your relationship quality is for you. It's really about the ability to really have empathy, to set boundaries, to resolve conflicts, to understand when a conflict is bad in a relationship, and to understand when it's an opportunity to grow. You also talked about communication, respecting boundaries, taking responsibility, forgiveness, intimacy, space. You said that to fall in love is one thing and building a relationship is another. But for you, what also differentiates couples with high relationship quality and those with your relationship quality is those other things are important. But for you, it's also really important to have curiosity, you know, to be curious about your partner, because again, it falls down to being seen, to being heard, to have an emotional resonance. So for you, that's important.

Therapist 44:12

And to add, I mean, it sounds easy, but curiosity is a skill you only can have when you're not traumatized. When you're traumatized, you're not curious, you're not curious about yourself, you're not curious about the other person. So it's not easy. Just be trust, be more curious. It goes much, much deeper. And it's also something you have to check. And to understand that that what is going on and this might be also something sometimes in couple sessions I can see that one of the clients might need more support in order to develop relationship skills, because if you really aren't unable to self regulate yourself, you cannot have a smooth relationship. In a way.

Interviewer 45:00

Interesting. All right. And then later on, we talked about whether how you conceptualize relationships differ from your clients. And here you talked about, you know, like or for therapy and how it impacts therapy. And for you, you were talking about, like for a therapist, you know, to be a good therapist also, they need to develop these skills in themselves. If the

therapist cannot forgive or a therapist can help people of always, etc., then how can they also teach that to others? And then we also talked about how you focus on which one to target in the complex system of a relationship. And for you, it depends. Sometimes it's what clients tell you. Otherwise it's, you know, as a gestalt therapist, it's the phenomenon of what you see in the room, what's conscious in the room. And then you also ask the couple what they think about it, because it should be important to the couple about what then you bring up. And then you end the couples therapy when either when you are satisfied or the client is satisfied or the goal has been achieved, or when they believe it doesn't work already. But usually this takes 10 to 15 sessions once every two weeks or once every month for 90 minutes. And then for you really you're interested in the skill. Might not use it yourself, but you think it's useful, you think it's interesting. And what you said that would be good for you. It is when it's in a like scale format, like 1 to 10 to make it easier for for people, for example, to, to maybe articulate or to put a representation to what they're feeling, which they might not they might be too stressed, they might not know how to name it, etc.. All right. Would there be anything you'd like to add to what we discussed? Now, now, before we end, I'd like to talk to you about how we will handle the data. Of course, as a research project, we want to be as transparent and as a verifiable as possible. So I want to know which data sharing option you're more comfortable with. One option is that only portions of the transcript can be shared, so none of it can be identified to you. Another option is that the whole transcript can be shared, removed of sensitive and overly identifying details. So it's still anonymized and people will have more context, for example, rather than just portions of the transcript. However, of course, in this option there is a chance that someone who knows you professional very well can piece together the details and might be able to identify. For example, oh, they know [Country] and if they professionally know you, they might be they might identify you through that.

Therapist 47:42

Is that data only your team?

Interviewer 47:45

No because it would be want it to be as open. So, for example, if we write the research manuscript and then we can link to an appendix. So if it were all the transcript portions. So for example, we say, oh, assessment, and these are the transcript portions that other people send. So it won't be linked to you or what they said about your relationship. So it will be.

Therapist 48:08

Actually, I don't care. You can use the second option because.

Interviewer 48:12

Really? I'll right.

Therapist 48:14

My opinion in the here and now. And it might be different in [1-5] years because I grew I don't know, because personal growth is what we need to do. Every human being, every therapist. So I'm happy to share the thoughts or the knowledge I have.

Interviewer 48:35

That's amazing. All right. I will be sending you the transcript If you want anything that we really redacted.

Therapist 48:48

That would be great.

Interviewer 48:44

One of these month, I might not be very happy. No one. Thank you so much. I will be ending the recording.